

# Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 02-Apr-2026

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## 1. Administrative & Study Identification

**Full Study Title:** A Multicenter, Phase 1, Open-label, Dose-escalation and Expansion Study of AZD0486, a Bispecific Antibody Targeting CD19 in Subjects with B-Cell Non-Hodgkin Lymphoma **Short Title/Acronym:** A Study of AZD0486 in Subjects With B-Cell Non-Hodgkin Lymphoma **Protocol Number:** D7400C00006 **NCT Number:** NCT04594642 **Sponsor Name:** AstraZeneca **Investigational Product:** AZD0486 (Surovatamig) - CD19xCD3 bispecific T-cell engager **Phase of Development:** Phase 1 **Medical Monitor:** AstraZeneca Clinical Operations Lead

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## 2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To evaluate the safety, tolerability, pharmacokinetics (PK), maximum tolerated dose (MTD), and determine the recommended phase 2 dose (RP2D) of AZD0486 monotherapy in subjects with B-cell non-Hodgkin lymphoma (B-NHL). **Secondary Objectives:** To evaluate the pharmacodynamics (PD), clinical efficacy (including Overall Response Rate [ORR], Complete Response Rate [CRR], Duration of Response [DoR], Progression-Free Survival [PFS]), measurable residual disease (MRD) clearance, and immunogenicity of AZD0486.

**2.2 Background & Rationale** To address the significant unmet medical need in patients with relapsed/refractory (R/R) B-NHL (including Diffuse Large B-cell Lymphoma [DLBCL] and Follicular Lymphoma [FL]) who have failed multiple prior lines of therapy, including CAR T-cell therapy. AZD0486 is an IgG4 fully human CD19xCD3 bispecific antibody designed with low CD3 binding affinity to minimize the severity of Cytokine Release Syndrome (CRS) while preserving effective T-cell cytotoxicity against malignant B cells.

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## 3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional (Dose-escalation and expansion study) **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A (Sequential assignment)

**3.2 Planned Enrollment & Duration Target \$N\$:** 317 subjects **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 02-Mar-2021 **Estimated Primary Completion:** 19-Apr-2028

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## 4. Subject Selection (Eligibility Criteria)

- 4.1 Inclusion Criteria**
1. Biopsy-proven B-NHL, including DLBCL, HGBL, or FL. (For R/R cohorts: must have received at least 2 prior lines of therapy and not be candidates for regimens known to provide clinical benefit).
  2. Subject must have locally confirmed CD19 positivity (documented after time of progression from last CD19-targeted therapy, if received) and at least 1 measurable disease site.
  3. Adequate organ function (eGFR  $\geq 50$  mL/min, ANC  $\geq 1000$  /mm<sup>3</sup>, platelets  $\geq 50,000$  /mm<sup>3</sup>, hemoglobin  $\geq 8.0$  g/dL) and an Eastern Cooperative Oncology Group (ECOG) Performance Status of  $\leq 2$ .
- 4.2 Exclusion Criteria**
1. Active central nervous system (CNS) involvement by B-NHL or clinically significant CNS pathology.
  2. Received CD19 CAR T therapy within 3 months prior to first dose, or experienced Grade  $\geq 3$  CRS or Grade  $\geq 2$  Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS) following prior T-cell engager or CAR T-cell therapy.
  3. Prior peripheral autologous stem cell transplant (SCT) within 12 weeks, allogeneic SCT within 1 year, or active/chronic infections with HIV, HBV, or HCV (exceptions for documented cure or undetectable viral loads on suppressive therapy).
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## 5. Intervention & Treatment Plan

- 5.1 Investigational Regimen Dosage/Frequency:** AZD0486 administered via step-up dosing schedule during Cycle 1 (e.g., priming doses on C1D1 and C1D8), followed by the target dose (ranging up to 25 mg) on C1D15. Subsequently administered every 2 weeks (Q2W) in 28-day cycles. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Fixed-duration treatment for up to 24 months (or until disease progression/unacceptable toxicity).
- 5.2 Concomitant & Prohibited Therapy Permitted:** Transfusion and/or growth factor support (provided counts are stable for at least 72 hours prior to screening). **Prohibited:** Other concurrent systemic lymphoma-directed therapies and ongoing immunosuppressive therapy post-SCT.
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## 6. Study Timeline & Visit Schedule

| Visit ID           | Window (Days) | Key Activities/Procedures                                                                         |
|--------------------|---------------|---------------------------------------------------------------------------------------------------|
| Screening          | Day -28 to 0  | Informed Consent, Medical History, CD19 Confirmation, Baseline PET-CT                             |
| Baseline / Cycle 1 | Day 1 to 28   | Inpatient administration for step-up dosing, safety monitoring (CRS/ICANS assessment), PK/PD labs |
| Interim            | Q2W           | Outpatient Target Dose infusions, Efficacy Assessment (PET-CT) every 12 weeks, MRD blood draws    |

**End of Study** | Up to Year 5 | Treatment completion (up to 24 months), Long-term survival follow-up, AE resolution |

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## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** Incidence of dose-limiting toxicities (DLTs) and treatment-emergent adverse events (TEAEs) to determine the MTD and RP2D. **Measurement Method:** Clinical safety evaluation using NCI-CTCAE version 5.0 (with ASTCT consensus grading criteria specifically used for CRS and ICANS).

**7.2 Secondary & Exploratory Endpoints** - Objective Response Rate (ORR), Complete Response (CR) rate, Duration of Response (DoR), and Progression-Free Survival (PFS) assessed by Independent Central Review (ICR) per RECIL criteria. - Attainment of undetectable Measurable Residual Disease (MRD) in plasma (e.g., within 12 weeks of treatment).

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## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Continuous monitoring with specific clinical protocols to grade and manage CRS, ICANS, cytopenias, and infections. **Serious Adverse Events (SAEs):** Reported per regulatory guidelines; strict DLT evaluation period executed during the initial 28-day cycle. **Data Monitoring Committee (DMC):** Internal Safety Review Committee (SRC) utilizing a modified toxicity probability interval (mTPI-2) design to safely govern dose-escalation phases.

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## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Safety Analysis Set (all subjects receiving  $\geq 1$  dose of AZD0486) and DLT-evaluable population. **Sample Size Justification:** Estimated  $N=317$ , statistically formulated by the mTPI-2 rules for dose escalation and adequately powered expansion cohorts to accurately estimate preliminary ORR and CR profiles. **Handling of Missing Data:** Patients with missing post-baseline efficacy assessments will generally be handled as non-responders for the ORR/CRR analyses.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote clinical monitoring, encompassing source data verification and comprehensive safety tracking. **Audit Trail:** Electronic Case Report Form (eCRF)

utilizing programmed edit checks, data clarification queries, and clinical database locks.

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## 11. Study Identifiers & Governance

**Primary ID:** D7400C00006 **Registry Number:** NCT04594642 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** A Study of AZD0486 in Subjects With B-Cell Non-Hodgkin Lymphoma **Official Regulatory Title:** A Multicenter, Phase 1, Open-label, Dose-escalation and Expansion Study of AZD0486, a Bispecific Antibody Targeting CD19 in Subjects with B-Cell Non-Hodgkin Lymphoma

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## 12. Clinical Framework (Oncology / Lymphoma Specific)

**Primary Condition:** B-Cell Non-Hodgkin Lymphoma (B-NHL) **Diagnosis Threshold:** Biopsy-proven R/R B-NHL with locally confirmed CD19 positivity **Medical Methodology:** Bispecific T-cell Engager (CD19xCD3) Therapy **Optimization Target:** Maximum Tolerated Dose (MTD) & Recommended Phase 2 Dose (RP2D)

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## 13. Study Procedures & Methodology

**Management Pathway:** Dose-escalation and expansion pathway for AZD0486 **Education Protocol (HCP):** CRS and ICANS ASTCT grading, monitoring, and management training **Education Protocol (Patient):** Inpatient observation requirements during Cycle 1 step-up dosing **Quality Audit Mechanism:** Independent Central Review (ICR) for PET-CT and continuous Safety Review Committee evaluations

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## 14. Observation & Follow-Up Schedule

**Total Duration:** Up to 5 years (Maximum 24 months of treatment + long-term survival follow-up) **Onsite Visit Cadence:** Cycle 1 (Inpatient/Intensive), then outpatient every 2 weeks (Q2W) **Remote Monitoring Frequency:** Routine survival follow-up check-ins post-treatment completion **Checklist Review Interval:** Disease response assessment (PET-CT) every 12 weeks

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## 15. Sign-off & Approval

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|  | Role                     |  | Name |               | Signature |  | Date |  |               | :--- |  | :--- |  | :--- |  | :--- |  |
|  | Principal Investigator   |  |      | [Insert Name] |           |  | ____ |  | [DD-MMM-YYYY] |      |  |      |  |      |  |      |  |
|  | Clinical Operations Lead |  |      | [Insert Name] |           |  | ____ |  | [DD-MMM-YYYY] |      |  |      |  |      |  |      |  |
|  | Lead Statistician        |  |      | [Insert Name] |           |  | ____ |  | [DD-MMM-YYYY] |      |  |      |  |      |  |      |  |